CKD Staging Reference (KDIGO-based)

Synthetic reference for demo purposes — general CKD concepts, not a clinical guideline or St. Luke's policy.

What counts as CKD

Chronic kidney disease is abnormal kidney structure or function present for **at least 3 months**: either an eGFR below 60 mL/min/1.73m², or markers of kidney damage (most commonly albuminuria), or both. A single low value during an acute illness or hospitalization does **not** establish CKD — use recent **outpatient** labs and exclude acute kidney injury.

eGFR (G) categories

- **G1** — eGFR ≥ 90 (normal/high; CKD only if damage markers present)
- **G2** — eGFR 60–89 (mildly decreased; CKD only if damage markers present)
- **G3a** — eGFR 45–59 (mild–moderate)
- **G3b** — eGFR 30–44 (moderate–severe)
- **G4** — eGFR 15–29 (severely decreased)
- **G5** — eGFR < 15 (kidney failure)

Albuminuria (A) categories (urine albumin-to-creatinine ratio, mg/g)

- **A1** — < 30 (normal to mildly increased)
- **A2** — 30–300 (moderately increased)
- **A3** — > 300 (severely increased)

Determining the stage

Average the recent outpatient eGFR values (at least three over ≥3 months), pair the eGFR (G) category with the albuminuria (A) category, and note a sustained downward trend. Higher G plus higher A means higher risk of progression and complications.